Olanzapine

Mechanism of action

- Olanzapine is an atypical antipsychotic.
- Olanzapine has actions at multiple receptors within the brain causing a reduction in agitation, sedation, anxiolysis and stabilisation of mood.

Authorised practice levels

- Paramedics, ICPs and CCPs.

Indications

- ✓ Patients aged greater than or equal to 12 years with acute behavioural disturbance causing a mild to moderate risk to safety, when the patient will take an oral medicine.

Contraindications

- ✕ Known severe allergy.
- ✕ Poisoning with an antipsychotic, for example quetiapine, risperidone or olanzapine.

Cautions

- ⚠ Pregnancy.
- ⚠ Intoxication. This will increase and prolong the effects.
- ⚠ Aged greater than or equal to 75 years, particularly if frail. The effects of olanzapine will be increased and prolonged in this cohort.

Use in pregnancy or when breastfeeding

- Safety has not been demonstrated during pregnancy and the balance of risk is such that olanzapine should usually be withheld.
- May be administered if the patient is breastfeeding. Advise the patient to stop breastfeeding and seek further advice from their lead maternity carer or GP.

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Dosage

- 10 mg PO.
- Consider reducing the dose to 5 mg PO if a caution is present.
- The dose may be repeated once after 20 minutes.

Administration

- Administer PO. The tablet is dispersible and will dissolve in the mouth. A sip of liquid will aid absorption but is not routinely required.
- The tablet may be dissolved in liquid. For example, water, tea or coffee.

Common adverse effects

- Sedation.

Usual onset of effect

- 10-20 minutes.

Usual duration of effect

- 12-24 hours.

Usual preparation

- 5 mg dispersible tablets (wafers).

Pharmacokinetics

- Olanzapine is predominantly metabolised by the liver.
- There are no significant effects from liver impairment on acute administration.

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Common interactions

- Intoxication. Olanzapine will have increased and prolonged effects if the patient is intoxicated with alcohol or has taken recreational drugs.
- Sedative drugs. Concurrent administration with other sedative drugs (such as midazolam or droperidol) will result in an increased and prolonged effect.

More Information

Version history

11 September 2023 1.0.4
Update: Indications updated.

22 November 2022 1.0.1
Update: Indications.